

Policy on Sterilisation and Disinfection of Instruments, Equipment and Devices

«Hospital Name»

Document control

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Prepared by	CSSD In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» provides adequate space and appropriate zoning in the CSSD, follows written guidance aligned with national/international standards for the full cleaning-packing-sterilisation-storage-issue cycle, conducts daily physical/chemical and at least weekly biological validation tests, implements a documented recall procedure on any sterilisation breakdown, and does not issue from a failed or recalled load.

1. Purpose

This policy explains how «Hospital Name» implements infection prevention through sterilisation and disinfection of instruments, equipment and devices: providing adequate CSSD space and zoning; following written guidance for every step of the cleaning-packing-sterilisation-storage-issue cycle; managing reprocessing of identified re-usable items; carrying out and documenting regular validation tests; and implementing the recall procedure when a sterilisation breakdown is identified.

IPC in clinical areas, support services, HAI-prevention bundles, and IPC surveillance are covered in separate hospital policies.

2. Scope

This policy applies to CSSD staff, nursing staff who handle sterile instruments and equipment in clinical areas, and all staff responsible for sterilisation, disinfection, storage and issue of instruments, equipment and devices at «Hospital Name».

3. Policy standards

«Hospital Name»'s CSSD has a suitable location, unidirectional flow, and defined zoning with separation of clean and dirty areas and sufficient space. The full cycle — cleaning, packing, disinfection/sterilisation, storage, and issue — is performed as per written guidance that is in consonance with national/international guidelines. Cleaning removes visible biological material and dirt. Spaulding's classification guides the level of disinfection. Shelf life is defined by packing material and mode of sterilisation. Flash sterilisation is used only in exceptional situations. Sterilised items are stored appropriately across the organisation. Instruments identified for re-use are listed; the number of re-uses and reprocessing method are defined, monitored, and patients are informed. Physical/chemical tests are performed daily and biological tests at least weekly; Bowie-Dick and leak rate tests are carried out; each load has a unique number. Written recall guidance exists; the recall procedure is implemented on any breakdown; a mock recall is run at least annually.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not issue an instrument, device or pack from sterile storage when validation of that sterilisation load has failed, or when a recall of that load or machine is in effect.
- Do not perform flash sterilisation routinely — use it only in exceptional situations when there is insufficient time to sterilise by the preferred method.
- Run physical/chemical tests on all sterilisation cycles daily, and biological tests at least weekly.
- Do not issue sterilised items without confirming the load number, content, and expiry date.
- Do not reprocess an item beyond its defined number of re-uses, and do not reprocess single-use items unless a documented, accepted method and risk assessment exist.
- Do not handle used instruments without adequate infection prevention measures in place, whether cleaning is done in CSSD or in a patient-care area.

5. What we do

5.1 Provide adequate space and appropriate zoning

«Hospital Name»'s CSSD (or designated sterilisation area) has a suitable location with a proper layout providing unidirectional flow and zoning. Clean and dirty areas are separated. Sufficient space is available to ensure sterilisation activities can be performed properly. The layout aims to have separate areas for receiving, washing, cleaning, packing, sterilisation, sterile storage and issue; this is the optimal arrangement. The CSSD In-Charge and the IPC Officer walk the zoning regularly.

5.2 Follow written guidance for cleaning, packing, disinfection/sterilisation, storage and issue

The full cycle is performed as per written guidance that is in consonance with national/international guidelines. The CDC Guideline for Disinfection and Sterilization in Healthcare Facilities and HSI guidelines are reference sources. Cleaning of used instruments/equipment/devices removes visible biological material and dirt; it is preferably done in the CSSD, but if done in patient-care areas, adequate IPC measures are in place. After cleaning, sets are prepared and packed using appropriate material. Spaulding's classification guides the decision on high/intermediate/low-level disinfection. Disinfection/sterilisation is performed as per written guidance. Flash sterilisation is used only in exceptional situations when there is insufficient time to sterilise by the preferred method. Sterilised/disinfected equipment and sets are stored appropriately across the organisation — not only in CSSD. Shelf life and expiry date of sterilised items are guided by the packing material used and the mode of sterilisation.

5.3 Manage reprocessing of instruments, equipment and devices

«Hospital Name» identifies instruments, equipment and devices that are suitable for re-use. The number of re-uses and the reprocessing method for each identified item are defined and monitored. Patients are informed about the re-use of items. Written guidance addresses cleaning, disinfection or sterilisation between patients for each reprocessed item. The written guidance is in consonance with available good practices. The CSSD In-Charge maintains the re-use register.

5.4 Carry out and document validation tests for sterilisation

Validation tests for sterilisation are carried out by accepted methods. Physical/chemical indicator tests are performed daily. Biological indicator tests are performed at least weekly. Engineering validations — including the Bowie-Dick tape test and leak rate test — are carried out. Each sterilisation load has a unique load number and content description. Where applicable, temperature, pressure and time-record charts are maintained. The CDC Guideline for Disinfection and Sterilization in Healthcare Facilities 2008 is a reference for validation methods. A load that fails any validation test is not issued.

5.5 Implement the recall procedure on sterilisation breakdown

The sterilisation procedure is regularly monitored. When a breakdown in the sterilisation system is identified, the written recall procedure is implemented: issue from the affected load/machine is stopped, items from that load (traced by unique load number, machine number, and date) are retrieved from all areas, users are notified, and items are reprocessed or discarded. The organisation uses a batch-processing system with date and machine number to enable effective recall. The recall procedure is verified through a mock drill at least annually.

6. Stop-work authority

Do not issue an instrument, device or pack from sterile storage when validation of that sterilisation load has failed, or when a recall of that load or machine is in effect.

Stop-work applies to issue from CSSD / sterile store. Immediate life-saving use of the only available item is documented and the CSSD In-Charge is told the same shift.

The person who stops tells the CSSD In-Charge and the Infection Prevention and Control Officer the same shift. Refusing to issue a failed or recalled load is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring the CSSD is adequately resourced, staffed, and operating as required by this policy.
CSSD In-Charge	Owns day-to-day implementation of this policy; maintains the processing guidance, re-use register, and load records; approves issue; leads recalls; receives stop-work escalations and notifies the IPC Officer the same shift.
Infection Prevention and Control Officer	Monitors CSSD processes in IPC rounds; receives stop-work escalations; brings sterilisation audit findings to the IPC committee.
Nursing Superintendent	Ensures nursing staff in clinical areas check expiry dates and load numbers of sterile items before use, and escalate any doubt about sterility.
Quality Coordinator	Audits this policy; holds training records, staff acknowledgements, and stop-work event logs.

8. Quality monitoring

Monitoring area	What is monitored
CSSD space and zoning	Regular IPC/CSSD round records confirming adequate space, unidirectional flow, and clean/dirty separation.
Processing guidance compliance	Load records confirming cleaning, packing, sterilisation, storage and issue per written guidance; Spaulding classification applied correctly.
Re-use register	Re-use register updated; patient information records for re-used items on file; no item issued beyond defined re-use count.

Validation tests	Daily physical/chemical test records; weekly biological test records; Bowie-Dick and leak rate test records; all loads with unique number and content description.
Recall procedure	Written recall procedure on file; annual mock recall record; records of any actual recalls with tracing and retrieval evidence.
Flash sterilisation	Flash sterilisation log confirming use only in exceptional documented situations.
Stop-work events	Stop-work events logged with trigger, action taken, and outcome.

9. Training and staff acknowledgement

CSSD staff and nursing staff who handle sterile instruments shall be familiar with the cleaning-packing-sterilisation-storage-issue cycle, the Spaulding classification, the validation test schedule, the re-use register and patient information requirement, the recall procedure, and the stop-work authority in this policy.

I have read the Policy on Sterilisation and Disinfection of Instruments, Equipment and Devices of «Hospital Name». I will follow the processes described, including the stop-work authority in Section 6.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to CSSD staff, nursing staff, the IPC Officer, department heads, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
CDC	Centers for Disease Control and Prevention
CSSD	Central Sterile Services Department
HISI	Hospital Infection Society India
IPC	Infection Prevention and Control
IPCO	Infection Prevention and Control Officer
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OT	Operation Theatre
PPE	Personal Protective Equipment
SSI	Surgical Site Infection

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
IPC.7.a	Commitment	Sections 3 and 5.1 address adequate CSSD space, suitable location, unidirectional flow and zoning, and clean/dirty area separation.
IPC.7.b	CORE*	Sections 3, 5.2 and 6 address written guidance for the full cleaning-packing-sterilisation-storage-issue cycle, in consonance with national/international guidelines; Spaulding classification; flash sterilisation restriction; shelf life and expiry; appropriate storage; stop-work trigger at issue of a failed or recalled load.
IPC.7.c	Commitment*	Sections 3 and 5.3 address identification of re-usable items, defined re-use count and reprocessing method, patient information, and written guidance for cleaning/disinfection/sterilisation between patients.
IPC.7.d	Commitment*	Sections 3 and 5.4 address daily physical/chemical tests, at least weekly biological tests, Bowie-Dick and leak rate tests, unique load numbers, and temperature/pressure/time charts.
IPC.7.e	Commitment*	Sections 3, 5.5 and 6 address written recall procedure, implementation on any breakdown, batch tracing by load/machine/date, and at least annual mock recall; stop-work trigger when a recall is in effect.

13. Required Records/Evidence Checklist

CSSD space and zoning — IPC.7.a

- CSSD layout diagram or floor plan showing unidirectional flow and clean/dirty area zoning.
- Regular CSSD/IPC round records confirming adequate space and clean/dirty separation.

Cleaning-packing-sterilisation-storage-issue guidance — IPC.7.b (CORE*)

- Written processing guidance covering all steps of the cycle, in consonance with named national/international guidelines.
- Spaulding classification decision table in the guidance.
- Flash sterilisation log confirming use only in documented exceptional situations.
- Shelf-life and expiry-date record per packing material and sterilisation mode.
- Load records showing sterilised items stored appropriately (not only in CSSD).
- Stop-work event log for any issue-from-failed-load events.

Re-use register and patient information — IPC.7.c (Commitment*)

- Re-use register listing identified re-usable items with defined re-use count and reprocessing method for each.
- Patient information records confirming patients were informed about re-use.
- Compliance monitoring records confirming no item is issued beyond its defined re-use count.

Validation test records — IPC.7.d (Commitment*)

- Daily physical/chemical indicator records for each sterilisation load.
- Weekly biological indicator records.
- Bowie-Dick tape test and leak rate test records.
- Load log with unique load number, content description, and (where applicable) temperature/pressure/time chart.

Recall procedure — IPC.7.e (Commitment*)

- Written recall procedure naming: stop-issue step, retrieval method by load/machine/date, user notification path, and reprocess-or-discard decision.
- Annual mock recall exercise record with findings and any CAPA.
- Records of any actual recalls implemented, with tracing and retrieval evidence.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. IPC.7.
2. CDC Guideline for Disinfection and Sterilization in Healthcare Facilities, 2008. Centers for Disease Control and Prevention.

3. Hospital Infection Society India (HISI). Guidelines for Central Sterile Services Department.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked [Hospital to define] must be replaced with the hospital's own decision; a document issued with those markers left in place is not an approved policy.

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